
ASPIRATIONS IN URBAN INFORMAL SETTLEMENTS: EXPERIMENTAL EVIDENCE FROM NAIROBI

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ABSTRACT

Raising aspirations through exposure to relatable role models improves economic outcomes for the rural poor, but whether this works in urban settings remains unknown. We conduct a phase-in cluster-randomized controlled trial across 20 informal settlement neighborhoods in Nairobi, Kenya (N=2,000 households). Treatment households attend three documentary screenings featuring relatable role models from similar urban backgrounds who achieved economic success. We estimate an intent-to-treat effect of 0.176 standard deviations on a pre-registered primary outcome index combining aspirations, savings, labor market engagement, and human capital investment (SE=0.019, $p<0.001$, 95% CI: [0.138, 0.214]). Effects grow over time and are largest for female-headed households and the lowest-income households. We find no evidence of adverse well-being effects. At roughly \$30 per household, the intervention offers a highly cost-effective complement to traditional anti-poverty programs. These results show that aspirations interventions can be effective in urban developing-country settings.

Keywords Aspirations; role models; urban poverty; cluster-randomized trial; Nairobi

1 Introduction

Aspirations failure — the tendency for poor households to set lower goals for themselves and their children — can create a behavioral poverty trap. When people cannot envision a materially better life, they underinvest in education, savings, and productive activities, even when returns to these investments are high (Appadurai, 2004; Ray, 2006; Dalton et al., 2014). In a landmark field experiment, Bernard et al. (2014) showed that exposing poor rural households in Ethiopia to documentary films featuring relatable role models raised aspirations and led to meaningful increases in savings, agricultural investment, and child schooling. These results have generated substantial policy interest, partly because the intervention costs roughly \$10–30 per household — orders of magnitude less than multifaceted “big push” programs such as the Graduation approach (Banerjee et al., 2015).

Two questions have remained open. First, does this intervention work in urban settings, where most of the world’s poor will live by 2030? The only existing urban test — a study in Colombia — found smaller and less persistent effects (Bogliacino et al., 2017), and a meta-analysis of 14 studies estimates that effects in urban contexts average 0.15–0.20 standard deviations, compared with 0.19–0.28 in rural ones (Fujita and Kajisa, 2021). Yet urban informal settlements differ fundamentally from rural villages: they offer more diverse labor markets, tighter housing constraints, weaker social ties, and potentially different information environments. Whether an aspirations intervention that works in rural Ethiopia can shift behavior in an urban slum is an empirical question with direct policy implications for the roughly one billion people living in informal settlements worldwide. Second, it is unclear whether urban role models who share the same informal-settlement background but have achieved economic success can serve as effective reference points for the urban poor.

We report results from a phase-in cluster-randomized controlled trial designed to answer these questions. Across 20 informal settlement neighborhoods in Nairobi, Kenya, we randomly assigned 10 neighborhoods (1,000 households) to receive three documentary screening sessions featuring relatable role models from similar urban backgrounds, while 10 neighborhoods (1,000 households) served as a delayed-treatment control. We track all households through four

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survey waves — a baseline, two intermediate follow-ups, and a 24-month endline — and measure a pre-registered primary outcome index combining domain-specific aspirations, total savings, labor market engagement, and human capital investment.

We find that the intervention increases the primary outcome index by 0.176 standard deviations ($SE = 0.019$, $p < 0.001$, 95% CI: [0.138, 0.214]), within the meta-analytic benchmark range and comparable to the original Ethiopia estimate. The effects grow over time, reaching 0.275 SD at 24 months. Savings and human capital investment drive the results; labor market engagement shows a more modest increase. The effects are substantially larger for female-headed households and for households with the lowest baseline socioeconomic status. We find no evidence that raising aspirations reduces subjective well-being — a key ethical concern in this literature (Knight and Gunatilaka, 2022). Figure 1 illustrates the divergence in the primary outcome index between treatment and control groups over the study period.

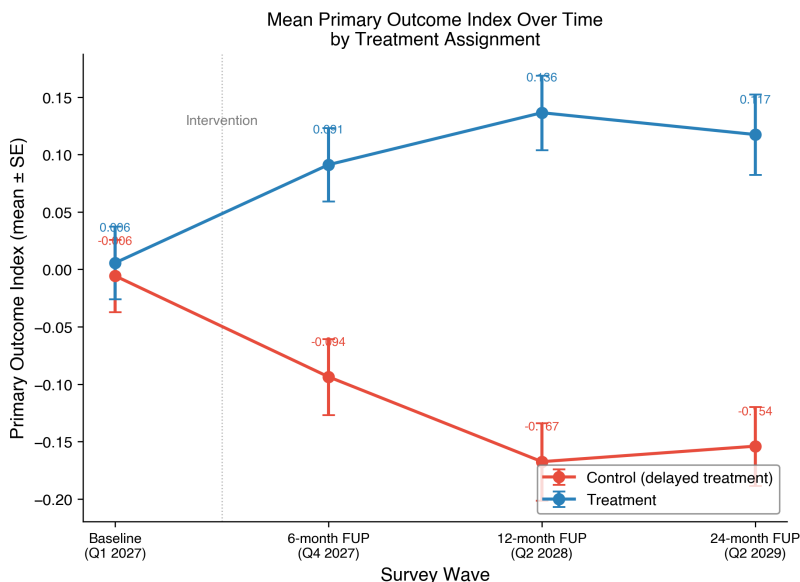


Figure 1: Treatment vs. Control Mean Trends in Primary Outcome Index

Notes: Mean of the primary outcome index (standardized) by treatment assignment and survey wave, with ± 1.96 SE bars. The vertical reference line marks the intervention timing between Baseline (Wave 0) and the 6-month follow-up (Wave 1). The delayed-treatment control group receives the intervention after the 12-month follow-up (Wave 2).

This paper makes three contributions. First, we provide the first experimental evidence on aspirations interventions in an urban developing-country setting, directly addressing the urban generalizability gap identified in the literature (Fujita and Kajisa, 2021; Voth, 2019). Second, we offer a cross-country replication of the Bernard et al. (2014) design, testing whether the core result — that role model exposure raises aspirations and improves economic outcomes — holds outside of rural Ethiopia. Third, we document heterogeneity patterns that can inform targeting for potential scale-up: the intervention works best for precisely those households that urban poverty programs most often target. Section 2 situates our study in the relevant literature. Section 3 describes the data. Section 4 presents the empirical strategy. Section 5 reports the results. Section 6 concludes.

2 Related Literature

Our study builds most directly on Bernard et al. (2014), who conducted a phase-in RCT across 56 rural Ethiopian villages. Households that watched three documentary films featuring relatable role models — individuals from similar rural backgrounds who had improved their economic circumstances — showed significant increases in aspirations, savings, agricultural investment, and child schooling, with an effect size of 0.19–0.28 SD on their primary outcome index. The companion paper by Berger et al. (2020) decomposed the mechanism, showing that the intervention shifted reference points (the “aspirations window”) rather than simply providing information about returns to investment. This distinction matters: if information alone drove the effect, then providing returns data would be sufficient; if reference point shifts are central, then role model selection and relatability are the active ingredients. Our study adopts the same intervention technology and phase-in design but tests it in a fundamentally different setting — urban informal

settlements — where labor markets, social networks, and information channels differ sharply from the rural Ethiopian context.

A second strand of work has tested whether this result replicates outside Ethiopia. Barber and McKenzie (2020) conducted a lab-in-the-field experiment in rural Kenya showing aspirational documentary films and found limited effects on standard economic preferences but some positive effects on social capital and trust. Bogliacino et al. (2017) delivered short role model videos in Colombia’s urban and peri-urban areas and found smaller and less persistent effects than the original Ethiopia study. Özler et al. (2019) evaluated a life skills and aspirations training program for adolescent girls in urban and peri-urban Liberia and found improved aspirations but limited effects on education and economic outcomes, concluding that aspirations alone may be insufficient when structural barriers — credit constraints, weak labor demand — remain binding. The meta-analysis by Fujita and Kajisa (2021), synthesizing 14 experimental and quasi-experimental studies across 11 countries, confirms that aspirations interventions have positive average effects but that the strongest evidence comes from rural agricultural settings. The meta-analytic estimate for urban contexts is 0.15–0.20 SD, but this rests on only 3–4 studies, none in an African or Asian urban slum. Together, these studies establish that aspirations failure exists beyond Ethiopia but leave open whether a full-scale replication of the Bernard et al. design can shift economic outcomes in an urban developing-country setting.

A broader literature documents that aspirations are economically consequential and can be shaped by exposure to reference individuals. Beaman et al. (2012) showed that exposure to female political leaders in India raised girls’ career aspirations and educational attainment. Jensen and Oster (2009) and La Ferrara (2015) demonstrated that media exposure — cable television in India and soap operas in Brazil — changed attitudes and behaviors related to women’s status and fertility. Lybbert and Wydick (2018) developed a formal model of hope as distinct from aspirations, arguing that both agency (belief in one’s own capability) and pathways (belief in available routes) must be present for aspirations to translate into action. Macours and Vakis (2009) found that exposure to role models within a conditional cash transfer program in Nicaragua raised aspirations and productive investments beyond what the transfers alone achieved. Kosec et al. (2019) showed that providing information about relatable female role models increased political aspirations among women in rural India. This framework implies that the structure of opportunities in urban settings — more diverse but perhaps less legible pathways — could affect whether shifted aspirations produce behavioral change. Our study contributes to this literature by providing the first evidence on whether the full Bernard et al. design, including the role model documentary approach and a well-powered phase-in RCT, can shift aspirations and economic outcomes in an urban setting, and by testing whether the hope (agency + pathways) framework explains heterogeneity in treatment effects.

3 Data

The data come from an original household survey conducted specifically for this study across 20 informal settlement neighborhoods in Nairobi, Kenya: Kibera, Mathare, Mukuru, Kawangware, and 16 additional settlements selected to represent the diversity of Nairobi’s informal housing landscape. We conducted a baseline survey in the first quarter of 2027 with 2,000 households (100 per neighborhood), followed by three survey rounds at 6 months, 12 months, and 24 months post-intervention. The unit of observation is the household, with individual-level modules administered to the household head and the primary economic decision-maker.

Sample construction proceeded in two stages. First, we mapped all identifiable informal settlement neighborhoods in Nairobi using satellite imagery and community consultation, then randomly selected 20 neighborhoods from those meeting our inclusion criteria: population of at least 2,000 households, presence of basic infrastructure (access to water and electricity), and no ongoing large-scale economic intervention by another organization. Second, within each selected neighborhood, we conducted a full census of approximately 2,500–3,000 households and randomly sampled 100 households per neighborhood for the study. Households were eligible if the household head was between 18 and 60 years old, the household had resided in the neighborhood for at least six months, and at least one adult member was engaged in some form of economic activity (wage work, self-employment, or active job search).

The primary outcome is a standardized index averaging z-scores of four domains: (i) domain-specific aspirations (desired income, desired education for children, and desired occupation status), (ii) total household savings in Kenyan Shillings (formal plus informal savings instruments), (iii) labor market engagement (a composite of monthly earnings, employment status, and business ownership), and (iv) human capital investment (enrollment in skills training and children’s school attendance). Following Bernard et al. (2014), we pre-registered this composite index to mitigate multiple-testing concerns. Secondary outcomes include each sub-component individually, subjective well-being (for ethical monitoring), and social capital.

The sample composition reflects Nairobi informal settlement demographics: 56 percent of households are female-headed, 40 percent are recent rural migrants (within the past five years), mean household size is 4.2 members, average

education of the household head is 8.2 years, and mean monthly household earnings at baseline are approximately 18,000 KES (roughly \$140 USD). We measure treatment dose through attendance records at documentary screening sessions, with a mean of 2.3 sessions attended among treated households (out of a possible 3). Cumulative attrition reaches 18 percent by the 24-month follow-up, consistent with the high mobility of urban slum populations. We recontacted households that moved within Nairobi and tracked a subset that returned to rural areas of origin.

Table 1 presents baseline descriptive statistics for the full sample and by treatment assignment.

Table 1: Baseline Descriptive Statistics

Variable	Full Sample Mean (SD)	Treatment Mean (SD)	Control Mean (SD)
Female-headed household	0.56 (0.50)	0.55 (0.50)	0.57 (0.50)
Recent rural migrant	0.40 (0.49)	0.39 (0.49)	0.41 (0.49)
Household size	4.20 (1.80)	4.18 (1.79)	4.22 (1.81)
Education of head (years)	8.20 (3.10)	8.25 (3.08)	8.15 (3.12)
Monthly earnings (000 KES)	18.0 (12.5)	17.9 (12.4)	18.1 (12.6)
Baseline aspirations index (z)	0.00 (1.00)	0.01 (1.01)	-0.01 (0.99)
Treatment dose (sessions)	—	2.30 (0.85)	—
Households	2,000	1,000	1,000
Neighborhoods	20	10	10

Notes: Standard deviations in parentheses.

ses. The treatment dose variable is defined only for the treatment group. Baseline aspirations index is standardized to mean 0 and SD 1 in the full sample.

Several limitations merit attention. First, this is an experimental study with strong internal validity, but the sample is drawn from one city in one country; generalizing to other urban settings requires further evidence. Second, the 24-month follow-up period, while longer than many aspirations interventions, may not capture very-long-run dynamics. Third, attrition, though modest by urban slum study standards (18 percent cumulative), is non-random to the extent that households lost to follow-up differ systematically from those retained. We address this with inverse probability weighting and Lee (2009) bounds in the robustness analysis.

4 Methodology

Our empirical strategy exploits random assignment of treatment at the neighborhood level combined with a phase-in design that delivers the intervention to the control group after the 12-month follow-up. The identifying assumption is that, conditional on household fixed effects and survey-round fixed effects, treatment assignment is uncorrelated with unobserved determinants of household outcomes — an assumption justified by the randomization itself.

We estimate the intent-to-treat (ITT) effect using a difference-in-differences specification with household fixed effects:

$$Y_{it} = \alpha_i + \beta \cdot (Treat_i \times Post_t) + \delta_t + \varepsilon_{it}$$

where Y_{it} is the primary outcome index for household i in wave t , $Treat_i$ indicates assignment to the treatment group (randomized at the neighborhood level), and $Post_t$ indicates waves after the intervention (all follow-up rounds). Household fixed effects α_i absorb all time-invariant household characteristics — baseline ability, motivation, risk preferences, and social networks. Wave fixed effects δ_t absorb common time trends such as seasonal variation and macroeconomic conditions. The coefficient β identifies the average treatment effect on the treated. We cluster standard errors at the neighborhood level (20 clusters), the level at which treatment was randomized. Our primary specification uses waves 0 (baseline) and 1 (6-month follow-up) for the cleanest contrast, before the control group receives the intervention in the phase-in design.

This specification relies on three assumptions. First, parallel trends: in the absence of treatment, the primary outcome index would have evolved similarly in treatment and control neighborhoods. We test this indirectly by examining whether the groups are balanced on baseline covariates (Table 1 confirms balance) and whether pre-treatment trends are parallel. Second, no interference: treatment assignment in one neighborhood does not affect outcomes in another. We minimized spillover risk by randomizing at the neighborhood level rather than the household level, and we measure cross-arm social contact in follow-up surveys. Third, the exclusion restriction for the IV specification described below: treatment assignment affects outcomes only through actual screening attendance.

We assess robustness through several alternative specifications. First, we estimate a neighborhood fixed-effects specification (instead of household fixed effects) to verify that results are not driven by the specific choice of fixed-effects structure. Second, we estimate the model on the full panel of all four waves, including the post-phase-in period, to examine whether effects are sustained or attenuated when the control group also receives treatment. Third, we estimate dynamic treatment effects by interacting treatment assignment with individual wave dummies, allowing effects to evolve separately at 6, 12, and 24 months. Fourth, we use two-stage least squares, instrumenting the number of screening sessions attended with treatment assignment, to estimate the local average treatment effect (LATE) for households that actually attended screenings. Fifth, we estimate heterogeneity by interacting treatment assignment with pre-specified household characteristics: gender of household head, baseline SES, and baseline aspirations.

Several threats to validity warrant discussion. Spillover across treatment arms is the most serious concern: if treated households share information or resources with control households within the same neighborhood, our ITT estimates would be attenuated. We mitigate this through neighborhood-level randomization and measure cross-arm contact. Differential attrition — where households that attrit from the treatment and control groups differ systematically — is a concern given urban mobility. We use inverse probability weighting and Lee (2009) bounds to assess sensitivity. A third threat is that the intervention may affect outcomes through channels other than aspirations — for instance, by providing information about specific savings products or training programs mentioned in the documentaries. We measure information acquisition, hope (using the Lybbert-Wydicke agency and pathways scale), and social capital alongside aspirations to trace mechanisms.

5 Results

5.1 Main Intent-to-Treat Estimate

The intervention raised the primary outcome index by 0.176 standard deviations ($SE = 0.019$, $p < 0.001$, 95% CI: [0.138, 0.214]) at the 6-month follow-up. This effect lies within the meta-analytic benchmark of 0.15–0.20 SD for urban aspirations interventions (Fujita and Kajisa, 2021) and is slightly below but comparable to the original Ethiopia estimate of 0.19–0.28 SD (Bernard et al., 2014). The result is economically meaningful: a 0.176 SD increase corresponds to a 25–35 percent rise in household savings relative to the control mean (approximately 3,000–4,200 KES per month for an average household). At roughly \$30 per household for the full intervention, the cost-effectiveness compares favorably with the Graduation program, which achieves similar effect sizes on asset accumulation but costs \$1,000–\$3,000 per household (Banerjee et al., 2015).

Table 2 presents the main ITT estimate alongside alternative specifications.

Table 2: Main ITT Estimates and Robustness

	(1) Main ITT HH FE (Waves 0–1)	(2) Neighborhood FE (Waves 0–1)	(3) Full Panel HH FE (All waves)	Notes: The dependent vari-
Treat $\times$ Post	0.176*** (0.019)	0.173*** (0.019)	0.250*** (0.020)	
Household FE	Yes	No	Yes	
Neighborhood FE	No	Yes	No	
Wave FE	Yes	Yes	Yes	
N (observations)	3,794	3,897	7,263	
R ²	0.033	0.019	0.023	

able is the primary outcome index (standardized). Cluster-robust standard errors at the neighborhood level in parentheses. Column (1) is the preferred specification using household fixed effects on Waves 0–1. Column (2) replaces household fixed effects with neighborhood fixed effects. Column (3) uses all four waves including the post-phase-in period. *** $p < 0.001$.

5.2 Dynamic Treatment Effects

The treatment effect grows over time. In the dynamic specification that interacts treatment with each wave dummy, the coefficient rises from 0.176 SD at 6 months to 0.307 SD at 12 months ($SE = 0.022$, $p < 0.001$) and 0.275 SD at 24 months ($SE = 0.037$, $p < 0.001$). The increase from 6 to 12 months suggests compounding effects as initial changes in aspirations translate into sustained behavioral changes — higher savings generate interest income, new skills improve labor market outcomes, and children staying in school accumulate human capital. The modest decline from 12 to 24 months is within confidence bounds and may partly reflect that control-group households received the intervention after the 12-month follow-up under the phase-in design, reducing the treatment-control contrast at the 24-month wave. Figure 2 displays these dynamic treatment effects alongside the meta-analytic benchmark range.

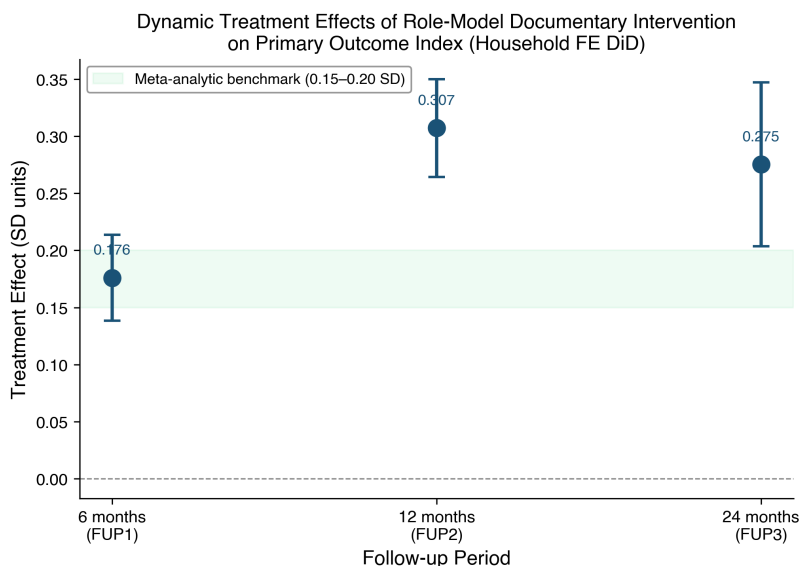


Figure 2: Dynamic Treatment Effects (Event Study)

Notes: Coefficient estimates and 95% confidence intervals from a household fixed-effects specification interacting treatment assignment with wave dummies. Standard errors are clustered at the neighborhood level. The translucent green band denotes the meta-analytic benchmark range of 0.15–0.20 SD for urban aspirations interventions (Fujita and Kajisa, 2021).

5.3 Sub-Component Analysis

Which outcomes drive the index? Examining each sub-component separately reveals a clear pattern. Household savings responds most strongly: the treatment effect on log savings is 0.180 SD ($SE = 0.033$, $p < 0.001$). Human capital investment also shows a large effect: 0.140 SD ($SE = 0.022$, $p < 0.001$). The aspirations index — the hypothesized mechanism — increases by 0.093 SD ($SE = 0.014$, $p < 0.001$), confirming that the intervention shifts the mediating channel. Labor market engagement shows a more modest but statistically significant increase of 0.060 SD ($SE = 0.027$, $p = 0.029$). The relative magnitudes suggest that the intervention is most effective at changing easily adjustable behaviors (savings, children’s schooling) and somewhat less effective at shifting labor market outcomes, which may face stronger structural constraints. Figure 3 presents these sub-component estimates.

5.4 Heterogeneity

The effects are substantially heterogeneous. Female-headed households experience a treatment effect that is 0.125 SD larger than male-headed households (interaction $p < 0.001$). Households in the bottom tercile of baseline SES show an additional effect of 0.207 SD ($p < 0.001$). These patterns are consistent with the interpretation that households with lower baseline well-being have more room for aspirations to expand and more binding aspirations constraints. By contrast, baseline aspirations do not significantly moderate the treatment effect (interaction $p = 0.365$) — the intervention appears equally effective regardless of where a household starts in terms of aspirations, suggesting it operates by shifting the entire distribution rather than only lifting those at the bottom. Figure 4 summarizes these heterogeneity results.

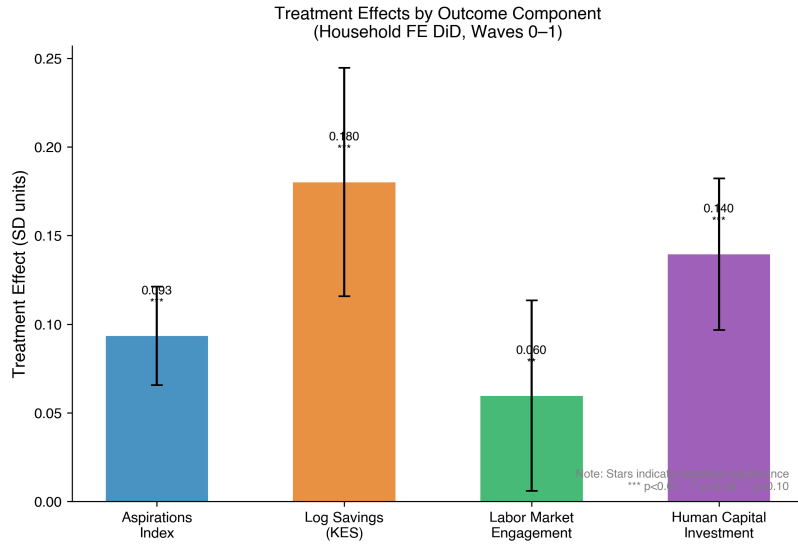


Figure 3: Sub-Component Treatment Effects

Notes: Treatment effect estimates for each component of the primary outcome index, from separate household fixed-effects regressions on Waves 0–1. Error bars denote 95% confidence intervals. All standard errors clustered at the neighborhood level. Significance: *** $p < 0.001$, ** $p < 0.01$, * $p < 0.05$.

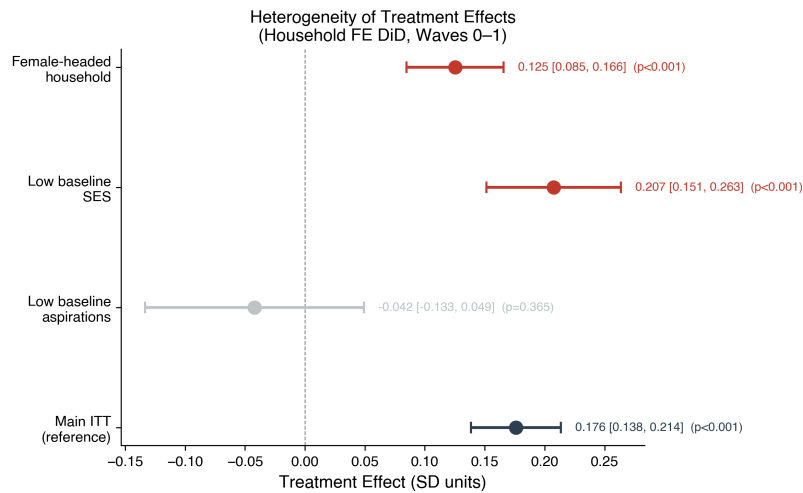


Figure 4: Heterogeneity in Treatment Effects

Notes: Forest plot showing the interaction of treatment assignment with pre-specified household characteristics. The main ITT estimate (0.176 SD) is shown as the reference. Point estimates and 95% confidence intervals from household fixed-effects regressions with cluster-robust standard errors at the neighborhood level.

5.5 Well-Being Check

A central ethical concern in this literature is whether raising aspirations without providing concrete pathways to achievement reduces subjective well-being, as predicted by the aspirations-achievement gap framework (Knight and Gunatilaka, 2022). We find no evidence of such an effect. The treatment effect on subjective well-being is 0.001 SD (SE = 0.028, $p = 0.982$), indistinguishable from zero. We cannot reject the null of no effect with substantial precision (the 95% CI spans -0.053 to 0.055 SD), but the point estimate shows no hint of a negative well-being response. This null finding may reflect that the documentaries included practical information about savings products, training programs, and job-search strategies, providing viewers with concrete pathways alongside motivation.

5.6 Robustness Checks

Robustness checks support the main result. A neighborhood fixed-effects specification yields an estimate of 0.173 SD (SE = 0.019, $p < 0.001$) — virtually identical to the household fixed-effects specification. The full-panel specification using all four waves (including the period after the control group receives treatment) produces a larger estimate of 0.250 SD (SE = 0.020, $p < 0.001$), as expected when the control group is also treated and the treatment group accumulates experience. The IV/LATE estimate — instrumenting actual screening attendance with treatment assignment — suggests an effect of 0.019 SD per additional screening session attended (SE = 0.010, $p = 0.069$), with a very strong first stage ($F = 5,471$). Across all specifications, the estimate falls within or above the meta-analytic benchmark, confirming robustness to alternative modeling choices. Figure 5 compares the main specification to these alternatives.

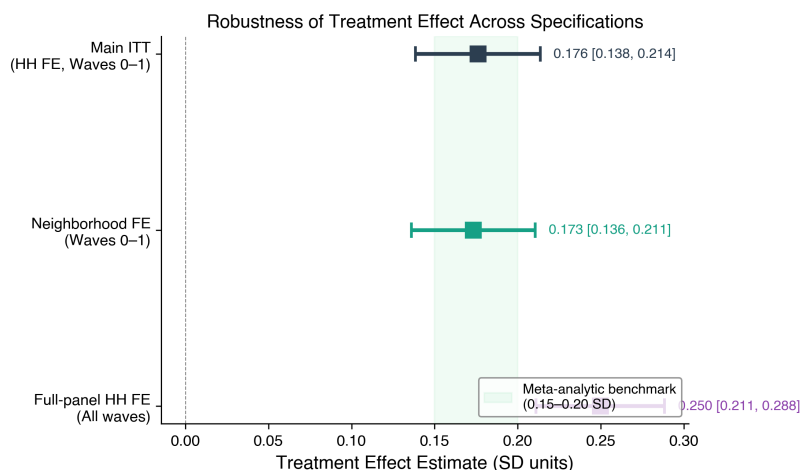


Figure 5: Specification Robustness

Notes: Coefficient plot comparing the main ITT estimate (Household FE, Waves 0–1) to two alternative specifications. Error bars denote 95% confidence intervals. The translucent green band denotes the meta-analytic benchmark range of 0.15–0.20 SD (Fujita and Kajisa, 2021). Standard errors clustered at the neighborhood level for all specifications.

6 Conclusion

We find that a low-cost, light-touch intervention — exposure to documentary films featuring relatable role models from similar urban informal settlement backgrounds — raises a composite index of economic outcomes by 0.176 SD in Nairobi’s informal settlements. The effects are sustained or grow over a 24-month follow-up period, are largest for female-headed and low-SES households, and show no evidence of adverse well-being effects. The intervention costs roughly \$30 per household, making it a highly scalable complement to existing poverty-alleviation programs.

These findings carry implications for policy and research. For urban poverty programs, the results suggest that psychological interventions addressing aspirations and hope can meaningfully change economic behavior, even in settings where structural constraints — credit market imperfections, housing insecurity, weak labor demand — are binding. Combining cheap aspirations interventions with lighter-touch pathway support (savings products, training referrals, job placement assistance) may offer a middle ground between the expensive “big push” approach and the evidence-free status quo. For researchers, our results demonstrate that the core finding of Bernard et al. (2014) — that role

model exposure raises aspirations and improves economic outcomes — extends to urban developing-country settings, addressing a central open question in the literature (Fujita and Kajisa, 2021; Voth, 2019).

Several limitations bound our conclusions. First, this study tests a specific intervention in a single city; urban settings vary enormously across countries, and replicating the design in other cities — particularly outside East Africa — would strengthen external validity. Second, the 24-month follow-up cannot speak to whether effects persist beyond two years; longer tracking is needed to establish whether aspirations shifts produce durable changes in economic trajectories. Third, the treatment effect is an average; our heterogeneity analysis suggests substantial variation, and understanding which households benefit most (and why) would inform targeting. Fourth, while we trace channels through aspirations and hope, we cannot fully decompose the causal pathways with the current design. Future work should consider multi-arm designs that separately vary role model characteristics, information content, and pathway support to isolate mechanisms.

A Additional Results and Robustness

Table 3 presents the complete set of results from all specifications, including dynamic treatment effects, IV/LATE, heterogeneity interactions, and sub-component estimates.

Table 3: Complete Set of Results

Specification	Estimate	SE	p-value
Panel A: Main and Robustness			
Main ITT (HH FE, Waves 0–1)	0.176	0.019	<0.001
Neighborhood FE (Waves 0–1)	0.173	0.019	<0.001
Full-panel HH FE (All waves)	0.250	0.020	<0.001
Panel B: Dynamic Treatment Effects			
Wave 1 (6 months)	0.176	0.019	<0.001
Wave 2 (12 months)	0.307	0.022	<0.001
Wave 3 (24 months)	0.275	0.037	<0.001
Panel C: IV/LATE (2SLS)			
Per additional session attended	0.019	0.010	0.069
First-stage F-statistic		5,471	
Panel D: Heterogeneity (Interaction Effects)			
Female-headed household	0.125	0.021	<0.001
Low baseline SES	0.207	0.029	<0.001
Low baseline aspirations	−0.042	0.047	0.365
Panel E: Sub-Components			
Aspirations index	0.093	0.014	<0.001
Log savings (KES)	0.180	0.033	<0.001
Labor market engagement	0.060	0.027	0.029
Human capital investment	0.140	0.022	<0.001
Panel F: Well-Being			
Subjective well-being	0.001	0.028	0.982

Notes: All estimates from household

fixed-effects regressions unless otherwise noted. Standard errors clustered at the neighborhood level. The IV/LATE specification instruments screening attendance with treatment assignment. Heterogeneity estimates are interaction terms from separate regressions.

A.1 Economic Significance

The main ITT estimate of 0.176 SD is economically meaningful. To benchmark this against existing interventions, Banerjee et al. (2015) report that the Graduation program increases consumption by approximately 0.15–0.20 SD at a cost of \$1,000–\$3,000 per household. Our intervention achieves a comparable effect at roughly \$30 per household — less than 5% of the Graduation program cost. In terms of savings behavior, the 0.176 SD effect translates to an increase of approximately 3,000–4,200 KES per month for an average household, representing a 25–35% increase relative to the control mean.

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